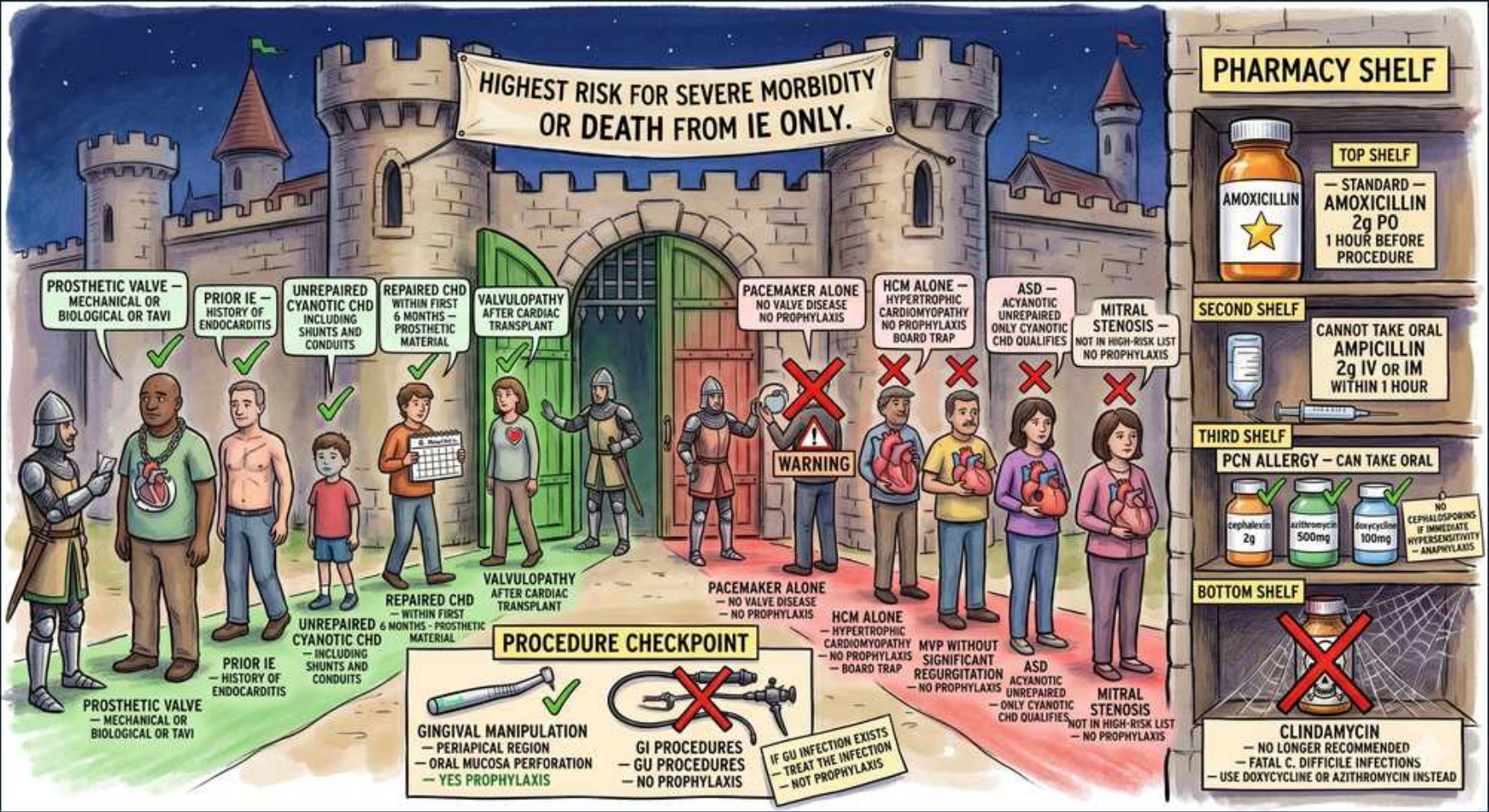


Infective Endocarditis — Prophylaxis: Who & What



1. High-risk principle

WHAT YOU SEE

Banner: highest risk for severe morbidity/death from IE only

WHAT IT ENCODES

Prophylaxis is reserved ONLY for cardiac conditions with the highest risk of severe morbidity/death from IE — a deliberately narrow list since universal prophylaxis showed no net benefit.

BOARD TRAP

Moderate-risk lesions (MVP, bicuspid valve, HCM) were dropped — prophylaxis is NOT for 'any structural heart disease'.

2. Prosthetic valve

WHAT YOU SEE

Knight with prosthetic valve placard

WHAT IT ENCODES

Any prosthetic heart valve — mechanical, bioprosthetic, or transcatheter (TAVI) — qualifies for prophylaxis, plus prosthetic material used for valve repair (annuloplasty rings/clips).

BOARD TRAP

TAVI and bioprosthetic valves count just like mechanical — don't restrict prophylaxis to mechanical valves only.

3. Prior IE

WHAT YOU SEE

History of endocarditis figure

WHAT IT ENCODES

A previous episode of infective endocarditis is an independent high-risk indication for prophylaxis regardless of valve type, given recurrence risk.

BOARD TRAP

Prior IE qualifies even with otherwise 'normal' valves — don't require a prosthetic to give prophylaxis.

4. Unrepaired cyanotic CHD

WHAT YOU SEE

Unrepaired cyanotic CHD / shunts placard

WHAT IT ENCODES

Unrepaired cyanotic congenital heart disease, including palliative shunts and conduits, is high-risk and warrants prophylaxis.

BOARD TRAP

It's the UNrepaired cyanotic CHD that qualifies indefinitely — fully repaired non-cyanotic defects generally do not.

5. Repaired CHD <6 months

WHAT YOU SEE

Repaired CHD within first 6 months

WHAT IT ENCODES

CHD repaired with prosthetic material/device gets prophylaxis only for the first 6 months post-procedure (endothelialization period), after which the risk falls.

BOARD TRAP

After 6 months with complete repair and no residual defect, prophylaxis is no longer needed — time-limited window.

6. Repaired with residual defect

WHAT YOU SEE

Valvulopathy after cardiac transplant / residual defect

WHAT IT ENCODES

Repaired CHD with a residual defect at/adjacent to a prosthetic patch, and cardiac-transplant recipients who develop valvulopathy, remain high-risk indefinitely.

BOARD TRAP

A residual leak next to prosthetic material keeps the patient high-risk beyond 6 months — defect matters.

7. Pacemaker alone — NO

WHAT YOU SEE

PACEMAKER ALONE / no valve disease / WARNING X

WHAT IT ENCODES

An isolated pacemaker/ICD (no valve disease) does NOT warrant antibiotic prophylaxis for dental procedures — a classic distractor on exams.

BOARD TRAP

Having a CIED alone is NOT a prophylaxis indication — board trap; only valve-related high-risk groups qualify.

8. HCM alone — NO

WHAT YOU SEE

HCM ALONE / hypertrophic cardiomyopathy X

WHAT IT ENCODES

Hypertrophic cardiomyopathy by itself is not on the prophylaxis list, even with outflow obstruction, unless there is a separate qualifying condition.

BOARD TRAP

HCM is a common wrong answer — outflow murmur does not equal prophylaxis indication.

9. MVP / regurgitation — NO

WHAT YOU SEE

MVP without significant regurgitation X

WHAT IT ENCODES

Mitral valve prolapse — even with mild regurgitation — is NOT an indication for prophylaxis under current guidelines.

BOARD TRAP

MVP is the single most common distractor — it does NOT get prophylaxis.

10. ASD — NO

WHAT YOU SEE

ASD acyanotic / repaired CHD X

WHAT IT ENCODES

Isolated secundum ASD, repaired ASD/VSD/PDA without residual defect, and other simple acyanotic lesions do not require prophylaxis.

BOARD TRAP

Simple ASD/VSD (no residual shunt) carries low IE risk — no prophylaxis.

11. Mitral stenosis — NO

WHAT YOU SEE

MITRAL STENOSIS X

WHAT IT ENCODES

Isolated native mitral stenosis (e.g., rheumatic) without a prosthesis or prior IE is not a prophylaxis indication under the narrowed criteria.

BOARD TRAP

Native valve disease (MS/AS) alone does not qualify — only the named high-risk groups do.

12. Gingival manipulation

WHAT YOU SEE

Procedure checkpoint: gingival manipulation / periapical / oral mucosa YES

WHAT IT ENCODES

Prophylaxis applies to dental procedures involving manipulation of gingival tissue, the periapical region of teeth, or perforation of oral mucosa.

BOARD TRAP

Routine dental cleaning without mucosal bleeding/manipulation may not require it — it's the invasive gingival work that counts.

13. GI/GU procedures — NO

WHAT YOU SEE

GI procedures / GU procedures NO prophylaxis

WHAT IT ENCODES

Routine GI and GU procedures (colonoscopy, cystoscopy) do NOT require prophylaxis. Only if active enterococcal infection exists, treat the infection — not as 'prophylaxis'.

BOARD TRAP

GI/GU endoscopy gets NO prophylaxis — a very common trap; antibiotics there are treatment of established infection.

14. Amoxicillin 2 g — top shelf

WHAT YOU SEE

Pharmacy shelf: amoxicillin 2 g PO 1 hour before

WHAT IT ENCODES

Standard regimen: amoxicillin 2 g PO a single dose 30-60 minutes before the procedure. Pediatric dosing is weight-based (50 mg/kg).

BOARD TRAP

It's a SINGLE pre-procedure dose, not a course — no post-procedure dosing needed.

15. Cannot take oral — IV/IM

WHAT YOU SEE

Cannot take oral: ampicillin 2 g IV/IM within 1 hour

WHAT IT ENCODES

If unable to take oral, give ampicillin 2 g IV/IM (or cefazolin/ceftriaxone 1 g) within 30-60 min before the procedure.

BOARD TRAP

Use ampicillin/cefazolin IV — not oral amox — when the patient can't swallow.

16. PCN allergy options / no clinda

WHAT YOU SEE

PCN allergy: cephalexin/azithro; CLINDAMYCIN X no longer recommended

WHAT IT ENCODES

Penicillin-allergic (non-anaphylactic): cephalexin 2 g. Cannot take oral: cefazolin/ceftriaxone. Doxycycline/azithromycin are alternatives. Clindamycin is no longer recommended.

BOARD TRAP

Clindamycin is OUT of current prophylaxis guidance (C. diff risk) — choosing it for PCN allergy is now wrong.